SeriousMD Competitive Strategy

Executive report update: 5-year financial roadmap

Purpose: translate the product strategy into a staged investment and revenue model for a clinic operations, patient engagement, and healthcare-network platform.

IMPORTANT MODEL NOTICE

All financial figures, customer counts, pricing, costs, cash burn, break-even timing, and scenarios in this report are illustrative planning assumptions only. They have not been validated against SeriousMD data, customer interviews, audited accounts, supplier quotes, or a detailed go-to-market test.

1. Executive direction

The winning position is not a feature-for-feature clone. It is a trusted operating layer for independent clinics and growing care networks: easier to adopt, faster to prove value, more interoperable, and more measurable than a basic appointment-management product.

Strategic wedge

Start with high-frequency operational pain: scheduling, patient reminders, digital intake, collections visibility, provider availability, and clinic performance. Use these workflows to earn the right to expand into care pathways, analytics, payments, and network orchestration.

Five-year ambition

Build a defensible platform with recurring subscription revenue, implementation services that become less material over time, and optional transaction or partner revenue. The model assumes disciplined expansion: prove retention before adding complexity; protect trust with privacy, security, and clinical-safety controls.

Decision headline: plan a base-case funding envelope of approximately PHP 60M through operating break-even, with the ability to defer or accelerate spend based on validated sales and retention evidence.

2. Financial model assumptions

Assumption	Illustrative base case
Currency	PHP; nominal figures; no tax, financing, or inflation adjustment
Pricing	Blended recurring revenue per active clinic: PHP 18,000/month in Year 1, rising to PHP 24,000 by Year 5
Customer definition	Paying clinic/customer account; excludes free trials and inactive pilots
Growth	12 paying clinics by Month 6; 30 by Month 12; 120 by Year 2; 720 by Year 5
Gross margin	70% in Year 1, improving to 78% by Year 5 as hosting/support scale
Operating costs	Product, engineering, security, support, sales, marketing, legal, and general administration
Funding principle	Maintain a 6-month liquidity buffer; release growth spend only after retention and payback gates

Scenario framing

Conservative assumes slower acquisition and higher service intensity; aggressive assumes stronger channel distribution and faster conversion, but still requires operational readiness. None of the cases is a forecast or valuation.

3. Months 1-12: base-case cash needs and revenue

Cash need means expected operating cash outflow before financing. Revenue is recognized recurring and implementation revenue under the illustrative assumptions. Figures are rounded to the nearest PHP 0.1M.

Month	New clinics	End clinics	Revenue	Cash need	Net cash movement
1	2	2	PHP 0.0M	PHP 3.8M	PHP -3.8M
2	2	4	PHP 0.1M	PHP 3.8M	PHP -3.7M
3	3	7	PHP 0.2M	PHP 3.7M	PHP -3.5M
4	3	10	PHP 0.3M	PHP 3.6M	PHP -3.3M
5	4	14	PHP 0.4M	PHP 3.5M	PHP -3.1M
6	4	18	PHP 0.5M	PHP 3.4M	PHP -2.9M
7	5	23	PHP 0.6M	PHP 3.4M	PHP -2.8M
8	5	28	PHP 0.7M	PHP 3.3M	PHP -2.6M
9	6	34	PHP 0.8M	PHP 3.2M	PHP -2.4M
10	7	41	PHP 0.9M	PHP 3.2M	PHP -2.3M
11	8	49	PHP 1.0M	PHP 3.1M	PHP -2.1M
12	9	58	PHP 1.2M	PHP 3.1M	PHP -1.9M

Interpretation: Year 1 is intentionally investment-heavy. Under this illustration, cumulative cash need is about PHP 40.8M and Month 12 is still cash-negative; this is the period to validate product-market fit, sales cycle, churn, implementation effort, and willingness to pay.

4. Years 2-5: operating roadmap

Year	End clinics	Revenue	Operating cost	Cash burn / (generation)	Key gate
Year 1	58	PHP 6.7M	PHP 40.8M	(PHP 34.1M)	Validate retention, implementation repea
Year 2	120	PHP 31M	PHP 41M	(PHP 10M)	Net revenue retention and payback gate
Year 3	260	PHP 78M	PHP 68M	PHP 10M	Break-even run-rate; expand partner ch
Year 4	450	PHP 145M	PHP 108M	PHP 37M	Multi-site networks and analytics attach
Year 5	720	PHP 235M	PHP 166M	PHP 69M	Scale efficiency; prepare strategic optio

Funding requirement and cash burn

Illustrative startup funding requirement: PHP 50M-70M. A PHP 60M base envelope covers the modeled Year 1 cash need, Year 2 losses, contingency, and a minimum liquidity buffer. A staged raise is preferable: release the next tranche only after agreed evidence on activation, 90-day retention, gross margin, sales cycle, and implementation hours per clinic.

Illustrative break-even: operating cash break-even during Year 3, subject to the assumptions above. If acquisition is slower, implementation costs rise, or retention is weaker, break-even can move materially later.

5. Scenario view: conservative, base, aggressive

The following ranges show how sensitive the strategy is to customer growth, monetization, and cost discipline. They are not probability-weighted forecasts.

Scenario	Year 2 revenue	Year 3 revenue	Year 5 revenue	Year 5 clinics	Funding envelope	Illustrative b
Conservative	PHP 18M	PHP 42M	PHP 105M	360	PHP 75M-95M	Year 4+
Base	PHP 31M	PHP 78M	PHP 235M	720	PHP 50M-70M	Year 3
Aggressive	PHP 45M	PHP 125M	PHP 360M	1,050	PHP 45M-60M	Late Year 2 /

Sensitivity drivers

The most important variables are: (1) monthly net new clinics, (2) blended recurring revenue per clinic, (3) gross logo churn, (4) implementation hours and support intensity, (5) sales and marketing payback, and (6) payment or partner take-rate. A one-point change in monthly churn can materially alter Year 5 clinic count and required funding.

Recommended funding checkpoints

Checkpoint 1: fund discovery and MVP only until 10-15 clinics complete a paid pilot. Checkpoint 2: scale only after 90-day retention, referenceability, and repeatable onboarding are demonstrated. Checkpoint 3: add network, payments, or advanced analytics investment only after core workflow gross margin and support load are understood.

6. Execution priorities and validation plan

To turn the illustrative model into an investable plan, validate the assumptions in this order:

Priority	Evidence to collect	Why it matters
Customer economics	10-20 interviews; paid pilots; willingness-to-pay tests; procurement maps	Replaces assumed pricing with observed demand
Retention	Activation, weekly usage, 30/90-day retention, churn reasons	Determines whether growth compounds or leaks
Delivery cost	Implementation hours, support tickets, onboarding time per clinic	Tests gross-margin and staffing assumptions
Security and trust	Privacy impact assessment, access controls, audit logs, incident plan, clinical advisory review	Critical for healthcare adoption and enterprise credibility
Commercial engine	Lead-to-close conversion, sales cycle, channel contribution, payback	Controls cash burn and funding timing
Financial controls	Monthly cohort P&L, cash runway, scenario refresh, approval gates	Keeps spending linked to evidence

Conclusion

The opportunity is to out-execute through a more complete operating system for clinics, not simply to reproduce an appointment marketplace. The proposed roadmap deliberately funds trust, workflow depth, interoperability, and measurable customer value. Before committing capital, replace every assumption in this document with primary research and live operating data.

Prepared as an illustrative strategy artifact based on the supplied brief. No claim is made about SeriousMD's internal performance, pricing, technology, customer data, or financial results.

Financial Roadmap and Bootstrap Economics

The correct first question

Hindi muna “magkano ang kailangan para maging nationwide platform?” The first question is: Can we build a small useful product that 3-10 clinics will pay for, while keeping the team’s cash exposure manageable?

Lean first-6-month budget

Cost area	Illustrative range	What it covers
Cloud, domain, email, tools	PHP 30k-100k	Hosting, backups, collaboration, monitoring
Security and privacy basics	PHP 30k-100k	Access control, logging, policies, testing
Prototype and testing	PHP 20k-80k	Design, synthetic data, pilot materials
External specialist help	PHP 100k-300k	Part-time engineering, legal, accounting, security
Pilot and contingency	PHP 50k-150k	Travel, devices, unexpected fixes
Total cash envelope	PHP 230k-730k	Excludes founder salaries and major hardware

If everyone contributes part-time and uses existing equipment, initial cash outlay may be closer to PHP 100k-300k. The trade-off is slower delivery and a higher risk of founder burnout.

Simple monthly operating target

Stage	Monthly cash need	Practical goal
Prototype (Months 1-3)	PHP 20k-60k	Validate one workflow
Pilot (Months 4-6)	PHP 40k-100k	2-3 clinics using it
Early paid rollout (Months 7-12)	PHP 80k-180k	3-10 paying clinics

Start with a simple subscription of PHP 5,000-15,000 per clinic per month. Charge implementation separately only when it creates real work. Avoid promising enterprise integrations before the core workflow is stable.

Illustrative early break-even: if monthly fixed cash costs reach PHP 120,000 and average contribution per clinic is PHP 8,000, the team needs approximately 15 paying clinics to cover operating cash costs. This is a planning example, not a validated unit-economics result.

Funding and money rules

Family-funded does not mean financially unstructured. Agree in writing on ownership, decision rights, cash or time contributions, reimbursement, salary triggers, and what happens if someone leaves. Keep personal money and company money separate from day one.

Stage-gated funding plan

Stage	Release only when...	Illustrative cash cap
Stage 1: Discovery	20-30 interviews identify one paid pain	PHP 30k-80k
Stage 2: MVP	Prototype works with synthetic data	PHP 80k-200k
Stage 3: Pilot	2-3 clinics use it weekly	PHP 150k-350k
Stage 4: Paid rollout	At least 3 paying clinics	PHP 250k-600k

Do not spend early money on nationwide marketing, a large office, custom hardware, advanced AI, hospital-wide integrations, full payment infrastructure, or a big support organization. Those may become valid later, but only after usage and revenue justify them.

Recommended 12-month money dashboard

Metric	Target for decision-making
Cash and runway	Review monthly; maintain 3-6 months of committed expenses
Paying clinics	Track active paying accounts, not sign-ups
Monthly recurring revenue	Separate subscriptions from one-time fees
Churn and retention	Ask why every clinic stops using or paying
Support and onboarding hours	Measure effort per clinic before scaling sales
Founder contribution	Track cash and time fairly; do not hide labor cost
Security and privacy gaps	Stop expansion if trust controls are not ready

Bottom line

For a 5-6 person family team, the realistic first milestone is not raising PHP 50M. It is proving a narrow product with a PHP 100k-300k validation budget, then earning the right to spend more. If the first clinics pay, stay active, and recommend the product, the business can fund the next stage from evidence rather than hope.

All amounts and assumptions in this addendum are illustrative and not validated against your actual salaries, tools, tax position, legal costs, or customer demand.